



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : شربات عبد المحسن فارس

المحافظة : الجيزة الحي / المدينة : الواحات البحرية العنوان : قرية القصر

رقم البطاقة : 25910102101988 رقم التليفون : 01205698625 تاريخ تسجيل الإستمارة : 26-09-2026

العمر (تاريخ الميلاد) : 10-10-1959 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: شربات عبد المحسن فارسي
National ID: 25910102101988



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



شربات
عبدالمحسن فارس
قرية القصر
الواحات البحرية - الجيزة

١٩٥٩/١  ٢٥٩١٠١٠٢١٠١٩٨٨

LP1027491

٢٠٢٦/٠١ ٢٥٩١٠١٠٢١٠١٩٨٨

ربة منزل
أنثى
مسلمة
أرملة

البطاقة سارية حتى ٢٠٢٣/٠١/١٦




Patient's Name: Sherbat Abdel Mohsen Faris
Date: 2026-08-11
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Post Contrast Multi-Slice CT Scan of the Chest, Abdomen, and Pelvis:

I- CHEST:

- Left sided Subtle lingual atelectatic changes mostly representing a post-irradiation sequel.
- Both Lung Fields are showing no nodules, consolidation or collapse.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening, or masses were seen
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Noted thoracic aortic mild Atherosclerotic changes.
- Normal appearance of the chest wall.
- Noted Dorsal vertebral Spondylosis.

II- ABDOMEN AND PELVIS:

- Normal Sized liver showing homogenous parenchymal attenuation. But with noted hepatic segment VII focal calcified lesion measuring (1.5x1.2cm) ... likely is old Calcified granuloma, No other hepatic focal lesion or dilated biliary radicles with normal appearance of gall bladder.
- Both kidneys are normal site, average size, and showing a smooth, regular cortical outline and preserved parenchymal thickness and function.
- Noted right lower third ureteric large stone (2.3x1.3 cm) with right Mild Hydro-uretero-nephrotic changes
- Left kidney is showing No stones or back pressure changes.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, and IVC.
- The urinary bladder shows no evidence of gross masses, diverticulae, or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Noted Mild aortic & iliac Atherosclerotic changes
- Noted Lumbar vertebral Spondylosis.

Opinion:

- ✓ Noted hepatic segment VII focal calcified lesion measuring (1.5x1.2cm) ... likely is old Calcified granuloma for further follow up.
- ✓ Noted right lower third ureteric large stone (2.3x1.3 cm) with right Mild Hydro-uretero-nephrotic changes.
- ✓ Noted Dorsal & lumbar vertebral Spondylosis.
- ✓ No Appreciable pulmonary or Abdominopelvic Suspicious lesions.

Best regards,

Dr. Ayman Shawky, FEBR

Dr Ayman Shawky

Name : Sherbat Abdul Mohsen Faris

Date : 23-04-2026 10:15

Branch : Dokki

Medical No. : 357998

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعافك مستشفيات

ISOTOPIC BONE SCAN REPORT

- **Indication:** 66-year-old female with history of breast carcinoma. Evaluation for possible osseous metastatic disease.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection.

FINDINGS:

- No definite scintigraphic evidence of skeletal metastatic disease could be identified.
- Mildly enhanced tracer uptake at the distal shaft of the right tibia, likely traumatic, for correlation with clinical data.
- Exaggerated degenerative/arthritis changes are seen at different levels of the spine (more evident at the lower lumbar spine), sternum (as well as both sternoclavicular articulations), both shoulders as well as the weight-bearing joints.

IMPRESSION:

- No definite scintigraphic evidence of osseous metastases.
- Follow up as clinically warranted.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

ECHO REPORT

NAME: *مريم عبد الله*

DATE: 26/4/2026

ID NUMBER: /

SEX: ♀ AGE:

EF	60%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.3	2-4cm	LA	3.5	2-4cm
FS	28%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *Normal*

LV Wall Thickness *Normal*

LV Diastolic Function *low A wave (AP2)*

SWMA *No RWMA*

LA and Ao Root Dimensions *Normal*

Mitral Valve *Mild MR*

Aortic Valve *N*

Tricuspid Valve *trivial TR*

Pulmonary Valve

Right Side and Systolic Function *TAPSE: 2.3*

Intracardiac Masses and Effusion

DR.

[Signature]

03/01/2026

2

MDT FORM

Name:	SHARBAT Abd Elmohsen Faris	MRN:	7521
Age:	65 Y	ID:	
Menopausal Sta		Phone:	01225698625
NRS SCORE:		Marital Status:	
BMI:		Family History:	- VE
CO Morbidity:	HTN IHD	Reason Of MDT:	NEW

Clinical Data:

BX: IDC G3
ER: 0 PR: 0 HER2: +3 KI67: 60

Sonomammography:

RT BIRADS 5 ----- 6 O.CLOCK 5 CM from nipple fungating mass

Mets Workup:

thyroid enlargement retro sternal extension for throid profail

History:

Recommendations:

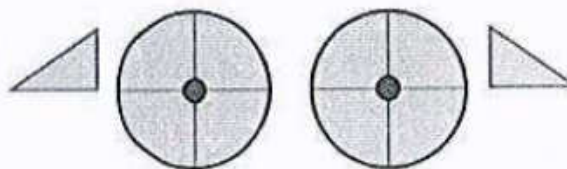
for NACT THEN RT MRM

MDT Outcome & Plan

FOR neoadj TCHP(with taxoter /carbo every 3 weeks)
then MRM After bone scan

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ حنان عزت	د/ احمد لبيب				

1081

Patient Name : الأستاذ / شربات عبدالمحسن فارس
Patient No. : 25910102101988
Path No. : 3900/25
Sex : Female
Age : 66 Y
Referring Doc. :

Request No. : 13004043
Request Date : 18/12/2025
Verification Date : 25/12/2025
Financial Category : جوستاف روسي - مبالغة
Department : Out Patient
IHC No. :
Registered by : Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10 % tumor cells (score 3+).
- Ki67: 60%

Conclusion:

Right breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade 3 with clear cell features & focal apocrine features.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: 60 %.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :

Dr.Heba Hasan

Printing Date : 28/12/2025 13:43:04

Page 1 of 2

Printed By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient Name الأستاذ / شربات عبدالمحسن فارسي
Patient No. 25910102101988
Path No. 3900/25
Sex Female
Age 66 Y
Referring Doc.

Request No. 13004043
Request Date 18/12/2025
Verification Date 25/12/2025
Financial Category جوستاف روسي - مبادره
Department Out Patient

Registered by Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

Printing Date : 28/12/2025 13:43:04

Page 2 of 2

Printed By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : sharbat abd al mohsen farouk

Date : 31-01-2026 12:52

Branch : Portable

Medical No. : GR1-25910102101988

Ref.Physician Prof : ct cap +c

Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of breast cancer for metastatic workup

I- CHEST:

- The right breast showed LIQ showed well defined soft tissue mass lesion measures about 3.5x3 cm in its maximum AP and TS dimensions respectively. associated with skin retraction and mild skin thickness measures about 9mm.
- Multiple enlarged right axillary lymph nodes, the largest is seen measuring about 4.6x2.5 cm. **for further sono-mammography.**
- Few enlarged left axillary lymph nodes, the largest measures about 1.94x1.55 cm
- Bilateral few scattered tiny pulmonary nodules are noted, the largest is seen in left lower lobar posterior segment measures about 3mm
- Right apical reticulation likely old granuloma.
- Bilateral basal atelectatic bands are noted
- Few enlarged right paratracheal lymph nodes, the largest measures about 1.2 cm.
- No pleural or pericardial collection.
- The heart is enlarged in size with atherosclerotic calcific foci of the aorta.
- Enlarged thyroid lobes with hypodense nodules and right retrosternal extension are noted **for further neck ultrasound.**

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
 - The left hepatic lobe segment IVa showed calcified lesion measures about 1 cm likely old granuloma.
- The gall bladder is well distended, average wall thickness with no stones.

Name : sharbat abd al mohsen farouk
Branch : Portable
Ref.Physician Prof : ct cap +c

Date : 31-01-2026 12:52
Medical No. : GRI-25910102101988
Ins.Company :

- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen (small splenule measures about 1.6x1.23 cm) , pancreas, right suprarenal gland and IVC.
 - Bulky left sided suprarenal gland is noted.
 - Atherosclerotic calcific foci of the aorta
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined skeleton no worrisome lytic or sclerotic lesions.

Opinion:

- Right breast LIQ soft tissue mass lesion with bilateral enlarged axillary lymph nodes being evident on right side for further sono-mammography.
- Bilateral few tiny pulmonary nodules likely nonspecific.
- Bilateral basal atelectatic bands.
- Cardiomegaly.
- Enlarged thyroid lobes with retrosternal extension for further neck ultrasound.
- Left hepatic lobe calcified lesion likely old granuloma.
- Bulky left suprarenal gland.

Dr.Norhan Ramzy,MD

Dr.NO

Women's Health Outreach Program

Name: Sharbat Abdel Mohsen

Date: 15-12-2025

Bilateral full field Digital MammographyClinical summary: 66-year-old right breast infra-mammary palpable lump and skin ulcerations.Technique

Medio-lateral oblique and CC views & High-resolution ultrasound

Detailed FindingsRight breast

- Scattered fibroglandular densities, (ACR b)
- Lower Central deeply seated partially obscured spiculated mass lesion with global skin thickening.
- No suspicious microcalcifications
- Ultrasound revealed a corresponding
 - 6 o'clock, 5 cm from nipple, infra-mammary speculated heterogenous hypoechoic irregular fungating speculated solid mass reaching the skin measured 41 x 30 x 20 mm.
- Large ?? two matted level 1 pathological lymph nodes measuring 45 x 20 mm. indeterminate lateral infraclavicular lymph node measuring 11 x 7 mm.

Left breast

- Scattered fibroglandular densities, (ACR b)
- No dominant mass, distortion or suspicious clusters of microcalcifications.
- Ultrasound: confirmed no solid or cystic lesion.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast: (BIRADS 5) Locally advanced malignant mass lesion , for US guided Tru-cut biopsy & histopathological assessment

Left breast: (BIRADS 1) Normal study

Best regards,
Dr. Marwa Hassan



Name: Sharbat adel Mohsen
Date: 15-12-2025

Women's Health Outreach Program

Clinical summary: 66-year-old, right breast infra-mammary palpable lump and skin ulcerations.

Technique

Medio-lateral oblique and CC views & High-resolution ultrasound

Detailed Findings

Right breast

- Scattered fibroglandular densities, (ACR b)
- Lower Central deeply seated partially obscured spiculated mass lesion with global skin thickening.
- No suspicious microcalcifications
- Ultrasound revealed a corresponding
 - 6 o'clock, 5 cm from nipple, infra-mammary speculated heterogenous hypoechoic irregular fungating speculated solid mass reaching the skin measured 44 x 30 x 20 mm.
- Large ?? two matted level 1 pathological lymph nodes measuring 45 x 20 mm.
- Indeterminate lateral infraclavicular lymph node measuring 11 x 7 mm.

Left breast

- Scattered fibroglandular densities, (ACR b)
- No dominant mass, distortion or suspicious clusters of microcalcifications.
- Ultrasound: confirmed no solid or cystic lesion.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast: (BIRADS 5) Locally advanced malignant mass lesion, for US guided Tru-cut biopsy & histopathological assessment
Left breast: (BIRADS 1) Normal study

Best regards,
Dr. Marwa Hassan

1053-11-2025
1053-11-2025

1053-11-2025
1053 Corniche El Nil St. For Al Khaleeg. Cairo

Name : sharbat abd al mohsen farouk
Branch : Portable
Ref.Physician Prof : et cap +c

Date : 31-01-2026 12:52
Medical No. : GRI-25910102101988
Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

A known case of breast cancer for metastatic workup

I- CHEST:

- The right breast showed LIQ showed well defined soft tissue mass lesion measures about 3.5x3 cm in its maximum AP and TS dimensions respectively. associated with skin retraction and mild skin thickness measures about 9mm.
- Multiple enlarged right axillary lymph nodes, the largest is seen measuring about 4.6x2.5 cm. **for further sono-mammography.**
- Few enlarged left axillary lymph nodes, the largest measures about 1.94x1.55 cm
- Bilateral few scattered tiny pulmonary nodules are noted, the largest is seen in left lower lobar posterior segment measures about 3mm
- Right apical reticulation likely old granuloma.
- Bilateral basal atelectatic bands are noted
- Few enlarged right paratracheal lymph nodes, the largest measures about 1.2 cm.
- No pleural or pericardial collection.
- The heart is enlarged in size with atherosclerotic calcific foci of the aorta.
- Enlarged thyroid lobes with hypodense nodules and right retrosternal extension are noted **for further neck ultrasound.**

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
 - The left hepatic lobe segment IVa showed calcified lesion measures about 1 cm likely old granuloma.
- The gall bladder is well distended, average wall thickness with no stones.

Name : sharbat abd al mohsen farouk

Branch : Portable

Ref.Physician Prof : ct cap +c

Date : 31-01-2026 12:52

Medical No. : GRI-25910102101988

Ins.Company :

- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen (small splenule measures about 1.6x1.23 cm) , pancreas, right suprarenal gland and IVC.
 - Bulky left sided suprarenal gland is noted.
 - Atherosclerotic calcific foci of the aorta
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined skeleton no worrisome lytic or sclerotic lesions.

Opinion:

- Right breast LIQ soft tissue mass lesion with bilateral enlarged axillary lymph nodes being evident on right side for further sono-mammography.
- Bilateral few tiny pulmonary nodules likely nonspecific.
- Bilateral basal atelectatic bands.
- Cardiomegaly.
- Enlarged thyroid lobes with retrosternal extension for further neck ultrasound.
- Left hepatic lobe calcified lesion likely old granuloma.
- Bulky left suprarenal gland.

Dr.Norhan Ramzy,MD

Dr.NO

Patient Name : الأستاذة / شربات عبدالمحسن فارس
Patient No. : 25910102101988
Path No : 3900/25
Sex : Female
Age : 66 Y
Referring Doc :

Request No. : 13004043
Request Date : 18/12/2025
Verification Date : 25/12/2025
Financial Category : جوماتك روسي - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data

Right breast 6 o'clock inframammary mass.

Nature of specimen

Tru cut biopsy, collected on 18/12/2025, at 1:00 p.m.

Gross examination

Multiple (4) grayish white tissue cores ranging in size from 0.5 up to 1.5 cm, totally submitted.

Microscopic examination

Histological type: Invasive duct carcinoma with clear cell features and focal apocrine features.

Grade (differentiation + nuclear pleomorphism + mitosis): (3+3+2) Grade 3

DCIS: Absent

Angiolymphatic invasion: Positive

Perineural invasion: Absent.

Diagnosis

Right breast inframammary mass, Tru cut biopsy:

Invasive duct carcinoma, Grade 3 with clear cell features and apocrine features.

For further IHC (ER, PR, HER2, Ki67)

Signature

Dr.Walaá Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

Printing Date : 28/12/2025 13:42:59

Page 1 of 2

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

1081

Patient Name : الأستاذة / شربات عبدالمحسن فارس
 Patient No. : 25910102101988
 Age : 3900/25
 Sex : Female
 Age : 66 Y
 Histology Doc. :

Request No. : 13004043
 Request Date : 18/12/2025
 Verification Date : 25/12/2025
 Financial Category : جوستاف روسي - مبادره
 Department : Out Patient
 IHC No. :
 Registered by : Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Referred Data: Right breast mass.
 Specimen: Tru-cut biopsy.

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
 (Positive control showed appropriate reactivity):
 ER: Positive nuclear reaction in tumor cells (Allred score 0/8).
 PR: Positive nuclear reaction in tumor cells (Allred score 0/8).
 Ki67: Strong complete membrane reaction in >10 % tumor cells (score 3+).

Diagnosis:
 Invasive duct carcinoma, Grade 3 with clear cell features & focal apocrine features.

ER: Negative (score 0/8).
 PR: Negative (score 0/8).
 Ki67: Positive (score 3+).

Staining system: "ER, PR"
 IHCAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0-3+): 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
 Dr.Heba Hasan

Date : 20/12/2025 13:43:04

Page 1 of 2

Printed By : .marwa mohamed yoi

Medical No.: 7691
Patient's Name: Sherbat Abdel Mohsen Faris
Date: 2026-09-26
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Pathologically proven right breast cancer, underwent NACT, follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- partial response of the lower central deeply seated mass with surrounding distortion and no suspicious microcalcifications.
- Thickened skin .
- **Ultrasound** demonstrated partial response of deeply seated pathologically proven malignant mass showing mild internal vascularity on color Doppler study was previously measuring measuring 44x 30 mm reaching the nipple that is seen retracted **it's measuring 2.8X6 mm in the current study .**
- thickened skin reaching 6 mm.
- Large two matted level 1 pathological LNS Measuring 15 mmx8mm and 7mm cortex the other one measuring 1.1 x 1.3 cm with 6mm cortex .
- No pathological infra-clavicular or supra-clavicular lymph nodes.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Evidence of benign macrocalcification.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

- **Right breast (BIRADS 6): Partial radiological response of the previously reported malignant process.**
- **Left breast (BIRADS 1): normal .**

Best Regards,

DR. Dina Sayed, M.Sc.CUDBIT

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS0: Non-informative study.	
BIRADS1: Normal study, routine screening	0 %
BIRADS2: Benign findings, routine screening	0 %
BIRADS3: Probably benign, for close follow up.	2 %
BIRADS4: Probably malignant, for biopsy.	50 %

A: mild suspicion.

B: moderate suspicion.

Medical No. : 7691

Patient's Name:

Sherbat Abdel Mohsen Faris

Date:

2026-09-26

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



BIRADS 5: Malignant, for biopsy.

98-100%

BIRADS 6: Pathologically proven malignant



09/20/2026

MDT FORM

Name:	SHARBAT Abd Elmohsen Faris محمد	MRN:	7521
Age:	67 Y	ID:	25910102101988
Menopausal Sta	post	Phone: *	01225698625
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:	HTN IHD	post nact	
Surgical History:*	NAD	ECOG Performance Status	0 1 2 3 4

Clinical Data:

BX:	IDC G3	R	L
ER:	0/8	PR:	0/8
HER2:	+3	KI67:	60%

History:

RT breast cancer her2 +3.
received NACT taxotere / carboplatin without phesgo 6c last 13/8/2026
with clinically improved
for your kind care and assessment

Sonomammography:

new sm >> requested

Biomarker/Molecular

Mets Workup:

Recommendations:

after examination good response to nact with still noted skin mark of the
initially fungating mass at the infra mammary line AT 6 OCLOCK for rt
mrm as planned

MDT Outcome & Plan

rt mrm as planned

TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
دكتور عبد البر	دكتور محمد كبري		